Memorial General Hospital - Department of Inpatient Medicine

CLINICAL DOCUMENTATION — NARRATIVE EMERGENCY OBSERVATION NOTE V1

Patient: Richard Martinez | **MRN:** PAT-60009 | **DOB:** 1955-05-17 | **Gender:** Male | **Doc ID:** NARRATIVE_0009
Encounter Date: 2025-04-06 | **Discharge/Note Date:** 2025-04-09 | **Provider:** Dr. Robert Evans, MD

EMERGENCY DEPARTMENT PRESENTATION

Patient Richard Martinez was triaged in the emergency department following acute onset of lightheadedness and palpitations earlier this morning. Initial hemodynamic parameters were reviewed and patient was placed on telemetry.

HISTORY OF PRESENT ILLNESS

The patient describes experiencing recurrent lightheadedness over the preceding 48 hours without syncope. Initial emergency workup revealed that initial platelet count was measured at 245.4 10*3/uL. Point-of-care laboratory screening additionally noted sodium resulted at 129.8 mmol/L (LOW). Given these findings, the patient was transitioned to the Clinical Decision Unit for 24-hour observation.

OBSERVATION UNIT CLINICAL COURSE

Throughout the observation unit stay, continuous telemetry showed normal sinus rhythm without pause or tachyarrhythmia. The patient was administered Omeprazole at a dose of 20 mg oral once daily before breakfast for 14 days, which promptly alleviated acute symptoms. Follow-up protocol monitoring of glucose demonstrated 88.1 mg/dL. Orthostatic blood pressures were negative upon ambulation.

DISCHARGE REGIMEN

Patient was deemed safe for outpatient discharge. The ongoing regimen includes Cephalexin 500 mg by mouth QID for 10 days as prescribed. Prescription counseling was performed by nursing prior to release.

PLAN & PRECAUTIONS

Resume normal daily activities as tolerated. Avoid strenuous exercise until cardiology follow-up in 1 to 2 weeks. Immediate emergency precautions detailed.